

**Staffordshire and Stoke-on-Trent Spirometry Service for Suspected COPD and or Asthma
Primary Care Referral Form**

Spirometry is a test which will help confirm clinically suspected asthma or COPD. Therefore, please ensure differential diagnosis is considered, before making this referral, ie other causes of cough; post nasal drip; reflux disease; suspected cancer, other causes of dyspnoea; cardiac disease; obesity; PE or other acute respiratory conditions.

Patient Details			
Hospital No:		NHS No:	NHS Number
Last Name:	Surname	First Name:	Given Name
Date of Birth:	Date of Birth	Address:	Home Full Address (single line)
Mobile No:	Patient Mobile Telephone		
Home Telephone No:	Patient Home Telephone		

Test(s) Required (please tick box to select)			
Suspected Chest Condition COPD and or Asthma (Review):	☐	This is a repeat spirometry	☐
	New Diagnosis:		☐

Clinical Details			
Reason for the test: Please tick.	COPD ☐ Asthma ☐ Other ☐ Please specify:		
Medication:	See summary below.		
Smoking Status:	Single Code Entry: Current smoker...		
If current smoker, years smoked	Free Text Prompt	Cigarettes / day:	Free Text Prompt
Vaping Status:	Single Code Entry: Ex-vaper...		
If ex-smoker, years quit:	Free Text Prompt	If ex-vaper, years quit:	Free Text Prompt

Spirometry Contraindications
Absolute Contraindications ● Active Lung infection e.g. AFB positive TB until treated for 2 weeks ● Active or suspected SARS-CoV-2 infection identified through PCR or LFD swab in the last 30 days, pre attendance questionnaire, increased temperature or presenting symptoms ● Conditions that may be cause serious consequences if aggravated by forced expiration e.g. dissecting / unstable aortic aneurysm, current pneumothorax, recent surgery including ophthalmic, thoracic abdominal or neurosurgery Relative Contraindications ● Suspected respiratory infection in the last 4-6 weeks ● Undiagnosed chest symptoms e.g. haemoptysis ● Any condition which may be aggravated by forced expiration e.g. history of prior pneumothorax; unstable vascular status such as recent (within 1 month) myocardial infarction, uncontrolled hypertension or pulmonary embolism or history of hemorrhagic event (stroke); previous thoracic, abdominal or eye surgery ● If the patient is too unwell to perform forced expiration ● Communication problems such as learning disability or confusion

Spirometry - General Information Note: A recent or current exacerbation/chest infection may impact on the accuracy of any baseline spirometry measurement, subsequently Bronchodilator would be needed. ☐ By selecting 'New Diagnosis' you are authorising the Lung Function team to administer 400mcgs salbutamol via a spacer as part of the test. ☐ By making this request, you are confirming you have individually assessed the patient and are confirming salbutamol is NOT contraindicated and can be administered. Health Care Professional Name: Bronchodilator response criteria ATS/ERS >12% and 200ml increase in FEV1 and/or FVC. Spirometry testing will include interpretation reported by a Health Care Professional. Interpretation will be done by an ARTP certified, or on an ARTP accredited course working towards accreditation. Airway obstruction will be classified using NICE COPD 2010 guidelines. FeNO - General Information The provider may carry out a FeNO test if considered necessary in addition to spirometry at the appointment. Use measurement of FeNO to find evidence of eosinophilic inflammation. A positive test increases the probability of asthma, but a negative test does not exclude asthma. FeNO testing will include technical interpretation to be reported by a suitably qualified clinician with appropriate training as per FeNO SOP. The Result Spirometry Complete - Advise the patient on agreed follow up with referring clinician. Spirometry unsuccessful - Leave patient on the waiting list to be rebooked at a later date. Technical report made and results added to patient record Requesting clinician notified with spirometry and or FeNO result and technical report to take action with the patient

Referrer Details			
Clinician Name:	Usual GP Full Name	GP Practice: Address:	Organisation Name Organisation Full Address (stacked)
Telephone Number:	Organisation Telephone Number	GP Practice Email:	Organisation E-mail Address
Referrer (Print Name):	Free Text Prompt	Referrer Signature:	Free Text Prompt
Date of Referral: Long date letter merged			
By signing this form, you have deemed the patient free from the above contraindications and to the best of your knowledge it is safe to proceed with the test.			

Problems
Problems

Medication
Medication

Tobacco consumption
Single Code Entry: Tobacco smoking consumption

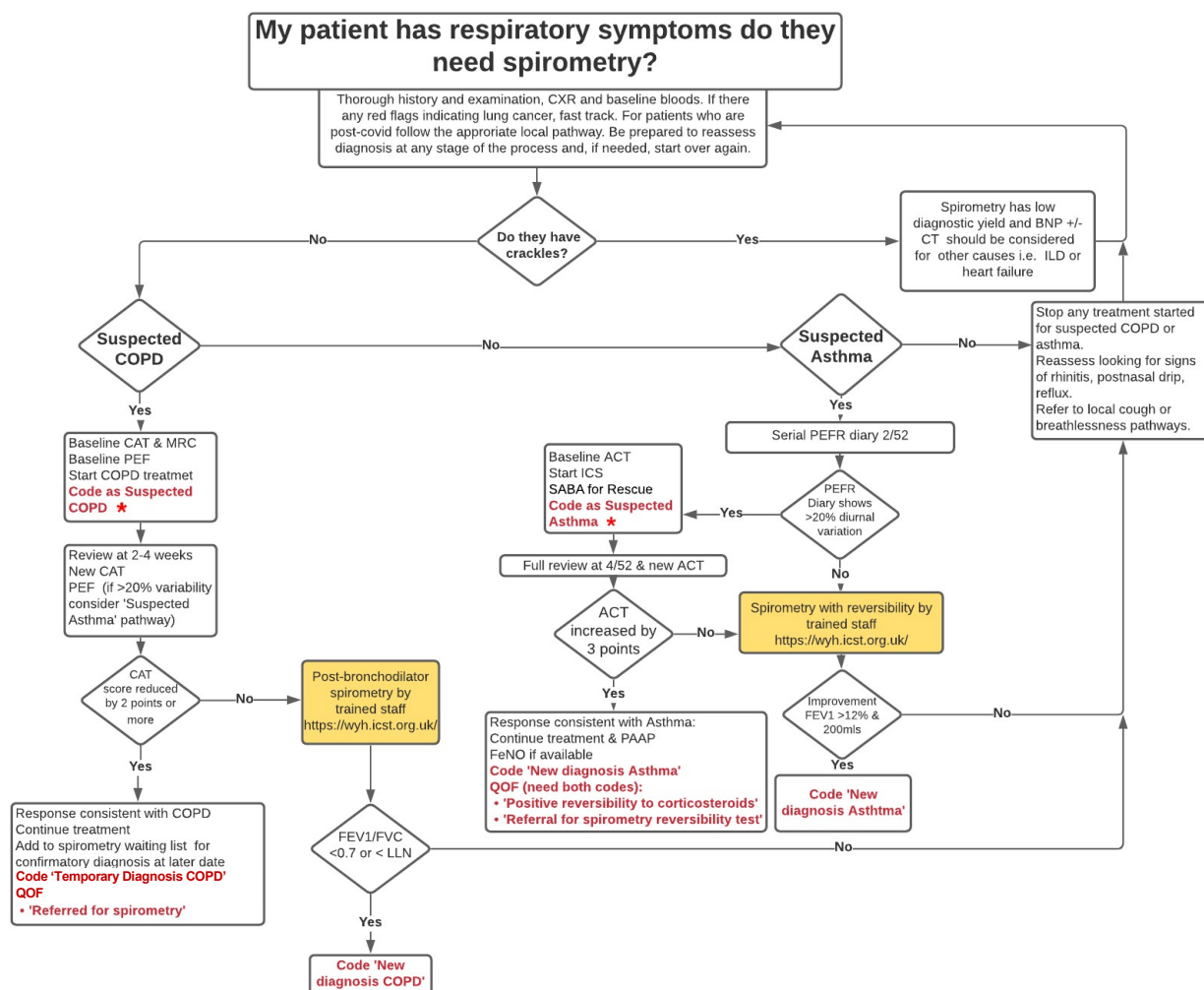
Investigations
Investigations

Prioritisation of Patient management before referral for Spirometry- "Does my patient need spirometry"

Since the COVID19 pandemic access to spirometry and reversibility testing has been severely restricted and resulted in a large number of patients receiving a temporary diagnosis and awaiting diagnostic spirometry.

The flow chart below is intended to be used to prioritise those patients who require access to treatment while waiting for spirometry testing. Consideration needs to be given to FeNO testing if there is any suspicion of asthma (please see FeNO SOP)

Please follow existing SSOT Asthma/COPD Diagnostic Guidance



*Please follow national guidance whenever possible.